3.16 Thoracic Surgery (Lobectomy and Pneumonectomy)*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "It hurts to breathe." **Objective** • Shallow breathing • Reduced air entry at the base • SpO_2 92%	**ACTUAL** Ineffective breathing pattern related to incisional pain and reduced lung volume as evidenced by shallow breathing and low SpO_2.	• SpO_2 94% or above with use of the incentive spirometer.	• Give regular analgesia, epidural or PCA as ordered. • Prompt hourly deep breaths and incentive spirometry. • Mobilize early. • Position as ordered; after pneumonectomy, avoid lying fully on the non-operative side.	• Met at target SpO_2.
Subjective • Rates pain 8/10 **Objective** • Guarding • Moves the arm on the operative side very little	**ACTUAL** Acute pain related to the thoracotomy incision as evidenced by pain score and guarding.	• Pain at or below 3/10 while doing arm exercises.	• Assess pain at regular intervals. • Teach shoulder range-of-motion exercises. • Show him how to splint the incision with a pillow when coughing.	• Achieved when pain is controlled and arm movement is full.
Subjective *No specific complaint* **Objective** • Major thoracic surgery • Chest drains	**RISK** Risk for excessive bleeding as evidenced by risk factors: major surgery and chest drains.	• Vital signs stable; drain output within expected limits.	• Measure drain output hourly in the early period. • Follow vital signs and hemoglobin. • Report heavy drainage.	• Outcome reached if there is no excessive bleeding.

*References for this care plan: [14], [32], [35], [37], [128]. Full entries are listed in the References section of the CPMN book.